

KING FAHD CENTRAL HOSPITAL (KFCH)

Comprehensive Stroke Program

POST-THROMBOLYSIS HEMORRHAGE CRITICAL ACTION CARD

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USE FOR SUSPECTED SYMPTOMATIC INTRACRANIAL HEMORRHAGE OR MAJOR BLEEDING AFTER IV THROMBOLYSIS

TRIGGERS	IMMEDIATE RESPONSE
- Acute neurological deterioration / NIHSS worsening - New severe headache - Acute nausea/vomiting or reduced consciousness - New marked hypertension with neurological change - Visible major bleeding or suspected internal bleeding	- STOP thrombolytic infusion immediately if still running. - Activate urgent Stroke/Neurology response and notify responsible physician. - Obtain emergency non-contrast CT head; do not delay for routine investigations. - Stabilize airway, breathing and circulation; treat life-threatening instability.

CRITICAL ACTIONS — PERFORM IN PARALLEL

Action	Completed / Time	Responsible
Urgent CT head (or appropriate emergency imaging) ordered and performed.	☐ _____	_____
Send CBC/platelets, PT/INR, aPTT, fibrinogen, type & screen/crossmatch, and other clinically indicated tests.	☐ _____	_____
Notify Stroke/Neurology and appropriate critical care team; notify Neurosurgery urgently for intracranial hemorrhage when indicated.	☐ _____	_____
Stop antiplatelet/anticoagulant agents and avoid non-essential invasive procedures until reviewed.	☐ _____	_____
Control blood pressure and other physiological derangements according to the approved KFCH hemorrhage/post-thrombolysis protocol.	☐ _____	_____
Initiate thrombolysis reversal/hemostatic treatment promptly when indicated, using the current KFCH Acute Stroke Complication Management Protocol/pharmacy-approved reversal pathway.	☐ _____	_____
Transfer to ICU/appropriate monitored setting when clinically indicated.	☐ _____	_____
Repeat imaging/laboratory assessment as directed by Stroke/Neurology/Neurosurgery/critical care team.	☐ _____	_____
Document event, treatment, times, outcome, and report as a medication/patient-safety event according to KFCH policy.	☐ _____	_____

REVERSAL / HEMOSTATIC THERAPY SAFETY CHECK

Reversal indicated by responsible physician?	☐ Yes ☐ No ☐ Pending
Specific agent/product and dose ordered	_____
Fibrinogen / coagulation reassessment ordered when applicable	☐ Yes ☐ No Time: _____
Blood bank / pharmacy contacted when required	☐ Yes ☐ No Time: _____

IMPORTANT: This action card intentionally does not hard-code a reversal product/dose. Use the current approved KFCH Acute Stroke Complication Management Protocol and pharmacy/blood-bank guidance, because reversal strategy depends on the thrombolytic used, timing, laboratory results, bleeding severity, and current institutional guideline.

EVENT DOCUMENTATION

Patient / MRN	_____	Thrombolytic agent	_____
Thrombolytic start time	_____	Deterioration/bleed time	_____
NIHSS before event	_____	NIHSS after event	_____
CT result	_____	Responsible physician	_____
Disposition	_____	Event report no.	_____

Document linkage: Supporting emergency action card to the KFCH Stroke Unit Policy & Procedure Manual 2026, IV Alteplase (tPA) Preparation & Administration Protocol, and Acute Stroke Complication Management Protocol. This card supports rapid response and does not replace physician judgment or the current approved protocol.